

Chapter 3: Health~4,000 words

Seen Tomorrow, Treated This Week: the aim - but is it realistic?

New Zealand is short of General Practitioners, "GP"s, the first port of call if you are ill or suffering a medical condition. Most GPs are booked up weeks in advance; the government intends to reduce that to one week for 80% of appointments "from 1 July 2026".ⁱ This, of course, is impossible, as we will see. In 2023, only 46% of waiting times were within a week.ⁱⁱ Additionally, appointments are typically limited to fifteen minutes, including time for the doctor to prepare prescriptions and write up the patient records.ⁱⁱⁱ If you need specialist treatment, the GP will arrange an appointment for you. But that will probably be months away.

New Zealand is short of Medical Specialists. Government aim to have most people requiring specialist attention, see them within four months^{iv}. But tens of thousand wait far longer, thousands more than a year.^v

Why? Because in 1982 New Zealand cut 50 places from the medical schools and although increased the number after 2004, now to double the 1982 number, but still the annual graduate number is at the low end of the international per population scale.^{vi}

How do we compare?

We don't compare badly — the whole world is suffering this crisis. The OECD "Health At A Glance" for 2025, notes that New Zealand had the lowest (under 3%) report of adults considering themselves to be in poor health. But we had the worst obesity rate, though lower than average smoking and air pollution. Netherlands had almost immediate access to a GP when needed, but otherwise to a practice nurse or other paramedic. In Netherlands virtually the whole country is covered by compulsory health insurance and the total cost of the visit is thus paid by the patient's compulsory insurance; the annual health costs per person are about 20% higher than NZ (which is at the OECD average). Although the ratio of doctors to population is virtually identical to NZ, thousands of Netherlands residents are on waiting lists for GPs.^{vii} Prices in Australia are almost twice those in New Zealand. Both Australia and New Zealand are largely (72%) government funded at about 20% of total government expenditure (Netherlands 16%). In summary, throughout the world (except in Netherlands, Switzerland and Germany) there are long wait times for consultation and surgery. New Zealand is little worse than the OECD average, except that the wait time here is rising,^{viii} as our population continues to grow (and age) faster than we train and recruit to replace our departing and retiring medical specialists.^{ix} This is The Pipeline Problem. It's why the government's 1 July 2026 target is unachievable.

The Pipeline Problem: the replacement cycle.

The replacement cycle starts at school when senior pupils decided to become doctors. At least 12 years later they become GPs, or 16 for other specialities. ASMS (Association of Salaried Medical Specialists) considers planning needs to be "two decades ahead to account for the time needed to train specialists" and that we have a current shortfall of about 1000. We have over 20,000 registered doctors, many working long past retirement, so we need 21,000.^x From the chart of Age of respondents the effective working life of a doctor is about 40 years. Then, approximately 1/40 will retire each year: 500 on average. The retirements will actually be greater, as the cohort has been growing older.^{xi} We graduate 600 per year but are already 1000 short. We could really only increase

our numbers by 100 per year if all our graduates were to stay here. However, at last count, four years ago, one in six New Zealand trained doctors worked overseas^{xii} that is, 100 per year. Each year 600 graduate, 500 retire and 100 go overseas. So our medical schools actually don't increase the doctor count at all.

Don't we import doctors? Yes, but is it a success?

Numerically, no. 70% or more of New Zealand-trained doctors are still here after ten years. Only 20% of International Medical Graduates, on average, are still here by that time, although those from Africa, Asia and the Middle East stay rather longer.^{xiii}

Why do so many immigrating doctors not settle in; and if we know, what can we do about it?

A recent NZ research paper,^{xiv} which includes a widely ranging references list, found that IMGs (International Medical Graduates) face language and social difficulties, in New Zealand as elsewhere. An IMG is permitted to practice here only after passing the New Zealand Registration Examination (NZREX Clinical for which only 66% — qualified in their own countries — pass) which also assesses 'cultural safety'. The difficulty apparently arises between the ability to describe these principles, and the difficulty in applying them in practice. Those whose native language is not English always have problems with English as a second language (ESL), and even after 70 years speaking only English, may still find some of our vernacular puzzling. The consequence is that IMGs frequently fail to appreciate the significance of what their patients say, and their comments to patients may fail to communicate the issues. Cultural differences are a continuing problem, not only in a non-medical environment, but particularly so in clinical practice and can lead to IMGs from a more formal, tightly supervised medical environment, here feeling abandoned, lacking supervision and guidance, as they struggle to become one of our more autonomous doctors. Though the IMGs may be aware of these issues and prepare for them, their NZ colleagues may not be, which can further exacerbate the problems.

There is no easy answer. Mentors have been suggested, but given the personal and professional difficulties the IMGs face, the retention numbers should not surprise us. Those who stay are probably those with most to gain, motivated to perseverance and tolerance, and with New Zealand representing their best option.

So, importing doctors is not a lasting solution for improving the health of anybody, let alone the disadvantaged.

New medical school — is that a solution?

No. Not enough. Not nearly enough.

The recently-begun Waikato Medical School is touted as the solution to our doctor shortage. So why won't it be enough?

Its first intake of 120 is scheduled for 2028, but it takes 12 to 16 years before each intake is fully qualified. 2040 to 2044. By then our population will be about 6.9 million and our need for doctors will have grown from 21,000 to 28,000, and the backlog from 1,000 to 8,000. That backlog, as our population grows, will be increasing at about 500 doctors per year. 120 graduates per year cannot halt the growth of backlog, and certainly can't reduce it.

We need medical schools with capacity for at least 500 intake per year, right now.

Otherwise we will find it increasingly hard to see a doctor — the disadvantaged, as those with reduced financial capacity, worst of all. More school capacity is desperately needed, now. We can afford it.

As things stand, the doctor shortage won't be fixed by 2056 — it will be much, much worse.

And our GPs: aren't they scarce too?

Yes, with 74 GP per 100,000 population we are below even the United Kingdom which is struggling with an inadequate service despite having 80 GP per 100,000 population, far below Australia (116) and Canada (122).

-

And the cost? Cheap compared to roading. Say \$1 billion for two schools. Roads? \$11 billion per year — and they don't cure anyone. One month of roading costs will fix our doctor shortage - eventually.

Support for the sick and injured: We must remove the anomaly of funding people's care differently depending whether they are laid low illness or injury. These people are suffering. They are not working, not caring for their family. By all means levy the employers to help defray the costs, but if we are to be a no-blame society, let's not distinguish between accident, which may have a blame-worthy cause, and sickness, which doesn't. The cost would be significant but not unquantifiable - the alternative is the current arbitrary distinction between a broken leg from a fall and a broken body from cancer.

-

- Dentistry: the forgotten health crisis
- What \$1 billion buys in health vs what \$11 billion buys in roads
- *Honest accounting: what 2056 actually means for people alive today*

- i Strengthening primary care: <https://www.beehive.govt.nz/release/strengthening-primary-care-better-meet-patient-needs>
- ii Figure 5.12 in: https://www.oecd.org/content/dam/oecd/en/publications/reports/2025/11/health-at-a-glance-2025_a894f72e/8f9e3f98-en.pdf
- iii Mornington Health Centre: <https://www.mhc.co.nz/News/2025/04/did-you-know-gp-appointments-are-15-minutes/>
- iv Scale of Govt's waiting list crisis: <https://newsroom.co.nz/2025/04/29/weekly-data-reveals-scale-of-govts-waiting-list-crisis/>
- v Wait times are a nightmare: <https://www.policywise.co.nz/resources/waiting-list>
- vi Forecasting New Zealand's future medical specialist workforce needs: <https://asms.org.nz/wp-content/uploads/2022/05/Research-Brief-specialist-workforce-projections-172060.2.pdf>
- vii Netherlands residents on waiting lists for GPs: <https://nltimes.nl/2024/03/21/thousands-netherlands-residents-lengthy-waiting-lists-primary-doctors>
- viii OECD (2025), *Health at a Glance 2025: OECD Indicators*, OECD Publishing, Paris, <https://doi.org/10.1787/8f9e3f98-en>. This volume, of 244 pages, https://www.oecd.org/content/dam/oecd/en/publications/reports/2025/11/health-at-a-glance-2025_a894f72e/8f9e3f98-en.pdf is an excellent comparative guide to most aspects of the cost and provision of health services throughout the OECD.
- ix New Zealand Doctor Shortage: <https://nzmj.org.nz/journal/vol-137-no-1592/the-aotearoa-new-zealand-doctor-shortage-current-context-and-strategies-for-retention>
- x Number of doctors in NZ: <https://www.mcnz.org.nz/assets/Publications/Workforce-Survey/Workforce-Survey-Report-2025.pdf>
- xi Age of respondents: https://www.rnzcgp.org.nz/documents/603/2024_WFS_snapshot_FINAL.pdf
- xii One in six NZ doctors works overseas: <https://theconversation.com/critically-understaffed-and-with-omicron-looming-why-isnt-nz-employing-more-of-its-foreign-trained-doctors-175914>
- xiii Retention by region: https://www.mcnz.org.nz/assets/Publications/Workforce-Survey/Workforce_Survey_Report_2024.pdf
- xiv Mannes, Thornley, Wilkinson (2023), "The consequences of cultural difference: the international medical graduate journey in New Zealand," *International Journal of Medical Education*. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC10693953/>